بسم الله الرحمن الرحيم

University of Shendi

Faculty of medicine and surgery

OSCE stations

1. Focus history.
2. Physical examination.
3. Procedures.
4. Counseling.
5. Data interpretation.

Paediatrics

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Focus history of malnutrition

Introduction
Great the pt,s
Introduce your-self
Take permission for history taking
Obtain the pt,s name &age
P/C:-
1) Diarrhea
2) Vomiting
3) Body swelling (generalized or localize)
4) Loss of weight
5) Respiratory distress
6) Refuse feeding

HPI:-
1) Analysis of diarrhea
2) Analysis of the vomiting
3) Analysis of swelling :-
4) When it start first
5) Course (static &progressive &fluctuating)
6) Relation of the swelling to the daily activity

Differential of generalized body swelling:
a. Renal origin
b. Cardiac
c. Liver malfunction
d. malnutrition

S/R:-
1) R.S (cough& SOB &hemoptysis)
2) C.N.S (convulsion)
3) U.S (dysuria & frequency &loin or supra pubic pain)

P M H:-
1) P M H of immunizable disease
2) Chronic illness:-
   a. C H D
   b. Congenital hypertrophy pyloric stenosis
   c. Giardiasis
   d. Congenital hyper trophy atrophic pyloric stenosis
3) Recurrent infection

Developmental history:-
Delayed milestone
1) Gross motor
2) Fine motor
3) Speech and language
4) Social and adaptive

Birth history:-
1. During pregnancy:-
   a. Anemia
   b. TORCHs infection
   c. Maternal malnutrition
   d. Maternal chronic illness
2. Natal ;-
   a. Prematurity
   b. Single or multiple
   c. Low birth weight
   d. Congenital malformation
   e. Neonatal infection
   f. Feeding difficultly

Nutritional history:-
What to feed:-
1) Breast milk
2) Artificial formula
3) Fresh milk

<u>Breast feeding :-</u>
- ✓ When did the breast feeding start?
- ✓ Exclusive for first 4-6 month or not ?
- ✓ If water was add is it boiled or not and how it applied?

- ✓ Dose the mother feed their child on demand or schedule?
- ✓ Weaning is it sudden or gradual?
- ✓ Practice of mother during diarrhea episode and febrile illness?
- ✓ Dose the mother known the technique of breast feed or not :-
  1. 10-15mitue for right breast
  2. Then left breast until the baby sleep
  3. Start the next feeding from the left breast to complete empting
- ✓ Number of meal/day?
- ✓ Was the infant calm and satisfied after feed or cries continuously before ,after, during meals
- ✓ Dos he sleep well after each meal (3-4hou
- ✓ Ask about breast milk (normal stool motion) no constipation or diarrhea (normal amount urine)

Artificial feeding (formula):-

- ✓ Why it artificial feeding?
- ✓ What is the type of milk?
- ✓ Average number of meal per day?
- ✓ Amount of milk in each meal?
- ✓ How does she prepare the meal?
  1. Amount of water and amount of milk powder dose she add sugar?
  2. Dose she use graduated bottle to preparation of meal or not?
  3. If fresh cow or buffalo milk ?dose she dilute it ?how much water added and dose she add sugar

Supplementary feeding:-

1. When start & is it sudden or gradual??
2. What type of food
3. Who much
4. How frequent given
5. How it applied
6. What about given eggs if not why

Vaccination history:-

Ask about the vaccination recommended to EPI or not (BCG scar)

F H:-

1. Mother
   a. Age
   b. Occupation
   c. Education
   d. Maternal pregnancy
   e. Maternal illness
2. Father :-
   a. Age
   b. Occupation
   c. Education
3. Family :-
   a. Size
   b. Spacing
   c. Similar condition
   d. Stability

S H:-

1) Housing condition
2) Social class
3) sanitation

Examination of the malnutrition

Introduction

General look:-

- ✓ Look ill or well
- ✓ Pallor
- ✓ Irritable or calm
- ✓ Distress or not
- ✓ Interested in surrounding or not

Vital sign:-

1) Pulse:-
   a. Tachycardia (infection or anemia)
   b. Rapid ()not palpable shock in diarrhea
2) BP :-
   hypotension severe diarrhea
3) Temperature:-
   a. Fever (infection)
   b. Hypothermia(complicated PEM)
4) RR:-
   a. Rapid shallow (pneumonia)
   b. Rapid and prolonged expiration(bronchitis)
   c. Rapid and deep (acidosis)in diarrhea

Anthropometric measurement:-

1) Weight
2) Height
3) Head circumference

Hair:-

1) Spared on temporal and occipital region
2) Dyspigmentation (gray or brown)
3) Dry or easily pick able
4) Flag sign

Fontanels

Depress in marasmus and marasmus kwash due to dehydration

Eye :-

1) Sunken (marasmus and dehydration)
2) Sign of vitamin deficiency
3) Jaundice ass hepatitis (fatty infiltrate)
4) Lack of tear(dehydration)
5) Keratitis (riboflavin deficiency)

Nose and ears :-

1) Rhinitis
2) Otitis media

Checks:-

1) Loss of bad fat old man face sever marasmus
2) Full doll like check(kwash)

Mouth:-

1) Pallor (anemia)
2) Angular stomatitis riboflavin

Tongue:-

1) glazed tongue(IDA)
2) thrush stomatitis
3) dry tongue(dehydrate)
4) delayed teething

neck:-

1) L N (TB .lymphoma. leukemia)
2) Goiter(loss of weight)

Extremities:-

1) Edema(kwash)
2) Wasting of muscle loss of subcutaneous fat (marasmus)
3) Cold and cyanotic extremities shock due to diarrhea

Skin:-

1) Loss of skin elasticity

2) Dermatitis (Kwash)

Chest examination ;-

To exclude :-

1) Pneumonia
2) TB
3) Empyema

Heart:-(precordium)

To exclude possibility of congenital heart disease

Abdominal examination:-

1) Hepatomegaly(fatty infiltrate)
2) Hepatospleenpmegily (TB. malignancy .lymphoma. metabolic disease)
3) Mass (neuroblstoma)
4) Ascites (Kwash)

C.N.S:-

1) To exclude meningitis
2) Psychic change (cardinal manifestation of Kwash)

Thanks the mother

Give the most likely diagnosis

DR yassir said the diagnosis must include

1) Classification
2) Present of life threatening condition
3) Nutritional deficiency
4) With infection or not
5) Risk of predisposing factor

1. Assessment of dehydration in malnutrition {DR Mohamed salah} by:-
   1) Skin pinch over the chest above the nipple
   2) Capillary refilling (thumb bed)
   3) Pulse

Lower limb examination

-introduction

-inspection :-

a- posture
b- wasting
c- fasciculation
d- trophic change
e- contraction
f- pes cava's

-palpation :-

a- muscle bulk
b- tenderness
c- hypertrophied peripheral nerve

-tone :-

a- normal
b- hypertonia
c- hypotonic

-power:-

a- symmetrical
b- proximal distal
c- along group muscle
d- along motor nerve distributions

<u>-Then examine:-</u>

a- hip:-
   - ✓ flexion (L1 ,2,3)
   - ✓ extension(L5,S1,2)
   - ✓ adduction
   - ✓ abduction

b- knee:-
   - ✓ flexion(L5,S1)
   - ✓ extension (L3,L4)

c- ankle:-
   - ✓ plantiflexion S1
   - ✓ dorsiflexion L5

-reflexes:-

a- ankle S1,S2
b- knee L3,L4

c- planter S1

-clonus{if hyper-refexia}:-

a- ankle

b- patellar

-coordination :-

a- toe finger test

b- heel skin test

-sensation :-

a- superficial:-

I. light pain

II. touch

III. temperature

b- deep:-

I. position

II. vibration

III. deep pain

-gait:-

1) gait
2) step length
3) step height
4) pelvic stability
5) associated movement

skull

-head size percentile
-head shape
-suture
-fontanel size
-sign of meningeal irritation
-spine

1) swelling
2) deformity
3) tenderness
4) trauma

Eyes examination

-Introduction
-Inspection:-

1) Position of eye lid
2) Symmetry of the eyes
3) Eyes brows
4) Lacrimal sac for swelling
5) Entropin
6) Extropin
7) Conjunctiva:-
   a. Redness ulcer scar
   b. Pallor
8) Sclera for jaundice
9) By the light from the lateral side inspect cornea and lens for opacity

-Optic nerve function :-

a. Visual acuity by simple finger counting method.
b. Visual felid
c. Ask for chart for colored vision
d. Request fundoscopy for fundus

-Extra-ocular muscle {conjugate eye movement}.
-Pupil examination

1) Size
2) shape
3) margin
4) equality
5) reaction to the light;-
   a. direct
   b. consenal
6) accomidation

CSF analysis

-macroscopic examination

a- pressure :-

2. 9-18 cmH_2O
    a. Normal
    b. Aspetic meningitis
3. 18 – 30 cm H_2O
    a. Acute bacterial meningitis
    b. TB meningitis
    c. Fungal meningitis

b- Appearance (color):-

1. Clear:-
    a. Normal
    b. Aspect meningitis
2. Cloudy:-
    a. others

-Cytology :-

a- RBCs

1. Normal (0-5cell/mm^3)
2. More than(5 cell/mm^3) indicate subarachnoid hemorrhage

c- WBCs:-

1. Normal 0-5 cell/mm^3 Lymphocyte
2. Acute bacterial (septic)meningitis 100-10,000 cell/mm^3 Neutrophil pre-dominant
3. TB meningitis (500cell/mm^3) Predominant lymphocyte then conversion from lymphocytosis to neutrophil
4. Fungal less than 300 cell/mm^3 predominate lymphocyte.
5. Aseptic less than 300 cell/mm^3 predominate lymphocyte.

-Biochemistry:

1. Glucose:
    a. Normal (2/3 of blood glucose = 50 – 75 mg/dl).
    b. Acute bacterial meningitis (marked decrease less than 45 mg/dl).
    c. TB meningitis (severely depressed glucose less than 45 mg/dl).
    d. Fungal meningitis (decrease less than 45 mg/dl).
    e. Aseptic meningitis (usually normal 50 – 100 mg/dl) but may be decrease.
2. Protein {Normal (20 – 40 mg/dl)}.
    a. increase acute bacterial meningitis.
    b. markedly increases TB meningitis.
    c. normal aseptic meningitis.

-Microbiology:

1. Gram stain:
    a. ±v for acute bacterial meningitis either gram stain positive or negative
    b. -ve for other type of organism.
2. ZN stain:
    +ve for TB meningitis and negative for other types of meningitis
3. Culture:
    a.

+ve for causative bacterial in the acute bacterial meningitis.

b. +ve in LJ media for TB meningitis.
c. +ve in special media in case of aseptic meningitis.

Birth history of CP

-Introduction
-Prenatal history :-
a- Maternal disease :-

1. Maternal age
2. PIH or HTN
3. DM
4. Maternal drug
5. Epilepsy
6. Maternal trauma
7. Fever and rash
8. Expose to radiation

b- About pregnancy:-

1. Is it multiple or single
2. Quality of fetal movement
3. Poly or oligo hydromainas
4. APH
5. ANC attendance

-Natal history :-

1. Is he or she term or not
2. Mode of delivery
3. Place of delivery
4. Duration of labour
5. Conducted by whom
6. Presentation of baby
7. Prolonged ROM
8. Difficulty in labour
9. Cord accident either prolapse or twisting around neck

-Post natal history :

1. Cry immediately after birth or need resuscitation
2. Start breast feeding or not and adequate
3. Meconium stain or not
4. Birth injury and birth weight
5. Abnormal feature
6. Association:-
    a. Convulsion
    b. Fever
    c. Jaundice

Hematological investigation in the pt stroke

RBCs count :-

- Poly cythemia> thrombosis >ischemia

Total WBCs:-

1. Neutrophil :-
    a. Brain abscess
    b. Meningitis
2. Lymphocytosis:-
    a. T.B meningitis
    b. Encephalitis
    c. Aseptic meningitis

Platelet count

1. Thrombocytosis (ischemic stroke)
2. Thrombocytopenia(hemorrgic stroke)

Coagulation profile

1. BT (low platelet +platelet dysfunction)
2. PT

3. APTT
4. Protein C+ S
5. Factor (viii)+WBF
6. Fibrinogen
7. Plasminogen

Investigation for SCA

1. Prephrail blood picture (sickle shape RBCs)
2. Sickling test
3. Hb electphoresis

Metabolic factor

1. Serum lipid
2. RBG
3. Urea and electrophoresis
4. Hyperhomocyteienimia
5. ABGs

Connective tissue decrease:

1. Anticardiolipin
2. ANA
3. Antidouble stranded AB
4. RF

Blood culture for SBE

Serum level of drug and toxins

Focus history of epilepsy

Introduction :-

Great the pt

Introduce yourself

Take permission for history taken

Obtain the pt name +age

Obtain the pt residence

P/C :-

Convulsion(duration)

HPI :-

1. Analysis of convulsion
    a. When it start
    b. Who notice it the convulsion
    c. Is it parietal or generalized
    d. Type of convulsion
    e. Duration each attack
    f. Frequency of attack
2. Events before attack :-
    a. Irritability
    b. Flash of light
    c. Noisy sound
    d. Crying
3. Event during attack :-
    a. Eyes rolling
    b. Tongue bite
    c. Incontince
    d. Loss of consciousness
4. Events after attack :-
    a. Headache
    b. Sleeping
    c. Vomiting
    d. Todd's paralysis
    e. Confusion
5. Complication:-
    a. Trauma

6. PPT :-
    a. Change in anticonvulsant drugs
    b. Change in the dose
    c. Tiredness
    d. Lack of sleep
    e. Fever
    f. Sensory stimuli :-
        i. Flashes
        ii. Television
        iii. Sounds

Review of CNS :-

1. Higher function
    (loss of consciousness)
2. Symptoms of increase ICP :-
    a. Plugging of fontanels
    b. Blurring of vision
3. Cranial nerve :-
    Deafness
4. Motor weakness
5. Any sensory impairment

PMH :-

1. Number of hospital admission
2. PH of similar condition
3. PH of diarrhea
4. PH OF hematuria
5. PH of DM
6. PH of immunization disease
7. PH of febrile convulsion

Birth history :-

1. Prenatal :-
    a. Fever
    b. APH
    c. Maternal trauma
    d. Exposure to radiation
2. Natal:-
    a. GA
    b. Duration of labor
    c. Trauma
3. post natal :-
    a. Crying at birth
    b. breast feeding
    c. infection

developmental :-

1. school of performance
2. millstone

V H:-

1. Measles
2. BCG
3. DPT

NH

1. Restriction of breast feeding
2. Diarrhea of weaning
3. Last meal ↓amount

F H:-

1. Similar condition
2. Febrile convulsion
3. Congenital malformation
4. DM HTN

D H :-

1. Miss dose
2. Change in drug
3. Side effects

SH :-

1. What restriction
2. School progress
3. Any social support
4. Hour medication
5. Family stability s

Addition :-

1. Skin rash :-
    a. Meningococcal septicemia
    b. Neurocutanous disease
2. Neck stiffness:-
    Meningitis

Thank the informant

Repeat your finding in summary

Counseling of convulsion

Introduction

Great the pt.

Take permission and introduce your self

Explain to the pt what is going

The management of convulsion is composed from two part :-

During attack:-

1. Don't run from the pt,s
2. Put the pt,s in lateral position to ensure the air ways were open
3. Remove any harmful object around the child
4. Observe the breathing and chest movement
5. Don't put anything in his mouth
6. Don't resist it
7. If the attack is prolonged call for help and carry his to hospital
8. Cold spongy if fever is present

In between attack :-

A- Removal of ppt

B- Medication and feeding of home member and school member

Removal of ppt:-

1. Avoid flashing
2. High pitch sounds
3. Television
4. Adequate sleep.

❖ Avoid attack in some situations:-

1. Pt doesn't for away from home mother
2. Avoid swimming
3. Don't play in over condition places
4. Avoid climbing of star trace
5. Avoid the driver of bicycle
6. Don't close the door of bath room behind him
7. Modification of life style

Learning about it medication and learning of school and family member :-

1. The medication should be taken regularly on specific
2. Regular follow up to change the dose according to it and follow up of complication
3. Learning the family and man over during attack in home or school
4. During the counseling you should explain everything going to say and check the understand of mother after each point

Summarize the point or ask the mother to summarize it to you

Thanks the pt,s his or his mother

Focus history hematuria

-Introduction
-Analysis of hematuria
1. Exact color of urine
2. Relation to micturition
3. Change in urine volume
4. Association

-Upper urinary tract
1. Fever rigor sweating
2. Abdominal pain
3. Burning micturition

-Lower urinary symptoms:-
1. Frequency
2. Urgency
3. Hesitancy
4. Weak stream
5. Retention
6. Urethral discharge

-Other :-
1. Buffy face
2. Lower limb edema
3. Bleeding for other site

-PMH :-
1. Similar condition
2. Sore throat
3. Bleeding disorder
4. Recurrent UTI
5. Urinary stone
6. PH of trauma
7. Schistosomasis
8. PH of skin lesion :-
    a. Infection
    b. Purpura
9. SCA
10. Deafness
11. Catheterization

-Nutritional history :-
1. Carrot ingestion
2. Protein restriction
3. Salt diet restriction
4. Any water limitation

-Family history :-
1. Similar condition
2. deafness
3. urinary stone
4. bleeding disorder
5. hemoglobinpathais

-drug history:-
1. rifampicin
2. NSAIDs
3. Steroid
4. Laxis
5. Anticoagulant
6. Cyclophosphamide
7. Methyldopa

Counseling of asthma

-introduction

-you have allergic inflammatory condition called asthma cannot be cured but it preventable Iam going to tell you about the prevention

- في حاجات بتهيج الازمة ولازم تتجنبها :

1- Dust and mites:-

1. يجب من الغبار نهائيا
2. تكيس المراتب والمخدات
3. تقلل من رطوبة الغرفة
4. تقلل من زحمة الاثاث في المنزل
5. الاعباب مكونات من النوع الفيو صوف انها لازم تكسح بالتراب

2- Upper respiratory tract infection :-

1. التهابات الجاري الصفنية العلوية لازم تشخص بدري وتعالج من بدري

3- Animals :-

1. يجب الطفل من المكان الفيو زحمة
2. من المفترض انو مابترو حيوانات في البيت لو ومن يجب هوتيها بعيد عن قدر المكان

4- Smoking :-

1. يجب عن أي نوع من الدخان لو كان لو كان حريق او غيرو
2. في زول يدخن داخل المنزل خاصة افراد الاسرة
3. لو كان من الدخان لابد من يجب هيوش يكون الطفل او الطفل في وجه عقب

5- Stronger perfume:-

لازم الطفل يجب عن الروائح القوية او تواليت بقدر المكان

6- Cock roaches:-

1. اول حاجة تقللوا كمية الصراصير في البيت بالمبيدات
2. تقلل من رطوبة الغرفة وتفيق الهيو والتخلص السليم من نهائيا الطعام لمنع البيت المائمة لتكاثر الصراصير

7- Drugs:-

في علاجات بتهيج الازمة زي الاسبرين ومضادات بيتلو لقدر الله الطفل مرض لازم تتبعي الدكتور انو عندو ازمة

8- Emotional stress and cold :-

1. تحاولي انو طفلك ده مافي حرمان ولا زعل شديد
2. تقللي من تعرض طفلك للبرد وتقلبات الجو

9- Exercise:-

التمارين مافي منعهو لكن يجب ان ياخذ العلاج قبل الرياضة (بخاخ)

Treat upper respiratory tract infection:-

1. Sinusitis
2. Rhinitis
3. Allergy

10- Treatment control :-

لازم ياخذ علاجات الازمة في الوقت المحدد وبالطريقة الصحيحة

11- How to use inhaler:-

a. Meter inhaler
b. Inhaler spacer
c. Powder spacer

-Ask the mother if has any question

History of chronic cough

-Introduction

-Analysis

1. Dry or productive
2. Day or night
3. Coming in episode or paroxysmal
4. Is it the pt of white sticky sputum
5. Are there any precipitating factor:-
    a. Fever
    b. Excersie
    c. Dust
6. Relation to posture
7. Quality of cough (soft or hash)
8. Is it complicated :-
    a. Hernia
    b. Hemorrhoids
    c. Rectal prolapse
    d. Hemorrhages
9. Association:-
    a. Fever
    b. Night sweating
    c. Loss of appetite
    d. Loss of weight
10. C P S:-
    a. S O B
    b. Palpitation
    c. Chest pain
    d. Hemoptysis
    e. Syncope attack

-P M H:-

1. Cardiac disease
2. Asthma
3. Chronic lung disease
4. TB
5. Contact with chronic cough
6. Chronic allergic sinusitis

-F H :-

1. Similar condition
2. TB
3. Bronchial asthma
4. Allergic

-SH :-

1. Socioeconomic class
2. Ventilation
3. Any smoker
4. Presence of chronic irritant

-D H :-

Anti tuberous therapy

Focus history of rheumatic fever

-Introduction
-About arthritis
1) Duration
2) Joint involve
3) Migration to other joint
4) Redness
5) Hotness
6) Limitation of the movement
7) Response to aspiration

-Carditis;-
1) Chest pain
2) Palpation
3) Sync opal attack
4) S O B
5) Cough
6) Hemoptysis
7) Lower limb edema

-Skin :-
1) Skin color
2) Skin lesion
3) Skin nodule

-Abnormal movement
-Minor criteria :-
1) Fever
2) Recurrent tonsillitis
3) Past history of RF

Counseling of DM nutrition

-Introduction
-Diet in form of 3 meals and 3 snakes
-Diet percentage as follow :-
- 55%CHO
- 15%protein
- 35%fat

-They should use road traffic diet :-
A- Red:_
- ✓ Chocolate
- ✓ Raw sugar
- ✓ Soft drinks
- ✓ Cake

B- Yellow:-
- ✓ Bread
- ✓ Milk
- ✓ Cereals
- ✓ Pasta

C- Green:-
- ✓ Meat
- ✓ Fish
- ✓ Soups

-Family should share child diet and don't let him eat alone
-Family should avoid the availability of the red diet at home as much as possible
-Diet is an important as insulin
-The need to involve nutritionlogest
-Diet should be enough to maintain appropriate growth
-No complete restriction
-Child should always carry one of sugar

2) الفخذين
3) الساعد
4) المؤخرتين

يجب تغير مكان الحقن بانتظام

-من الأعراض المضاعفات الحادة التي تحتاج لتدخل عاجل مع أخذ التاريخ المرضي :-

زيادة الجرعة أو أخذ الجرعة بدون الأكل يتبادر إلي الذهن نوبة نقص السكر ومن الأعراض التي تشير إلي حدوث نقص السكر في الدم

-المضاعفات المزمنة التي تحتاج لتدخل عاجل :-

1) زيادة الوزن
2) ظهور أعراض نقص السكر في الدم بعد الأكل مباشرة يجب تغير موضع الحقن

-الأكل المتزن يقلل من حدوث الهبوط الشديد

Counseling of hypoglycemia in diabetic pts

-Introduction
-Hypoglycemia can be confirmed by RBG less than 50 mg/kg
-I want to tell you more about the cause
-Common cause :-
1) Inadequate missed of delayed meal
2) Insulin over dose
3) Strong exercise
4) Infection
-Clinical presentation
a- Neuro glucopenic symptom:-
a- Headache
b- Lack of concentration
c- Blurring of vision
d- Convulsion
e- Coma

الأنسولين

-الانسولين عبارة عن هرمون يفرز بواسطة البنكرياس في الحالة الطبيعية وطبعاً في زول السكر يغير موجود عشان كده بنعطي انسولين خارجي وهو في الغالب مصدر حيواني لكن له نفس الوظيفة

-الانسولين في دخول الجلوكوز للخلايا لقلة انتاج البنكرياس من الهرمون

-طبعاً من أهم نوعين عندنا هو المخلوط والصافي (الصافي بيتخدم في الحالات الحادة تحت اشراف الطبيب المخلوط ده انتو بتستخدموه في البيت للتنظيم المستمر)

-وحدات الانسولين المطلوبة لتنظيم السكر يدار علي مدار اليوم بتعطي في الصباح واحدة بتاخد علي الصباح وده بتاخد واحدة بالمساء ده بتاخد تلت تلتين

-الجرعة لازم تتناول بانتظام في أوقات محددة من اليوم علشان السكر يفضل في حدود طبيعية والسكر لما ينزل عن الحد الطبيعي بيسبب نقص في الجلوكوز في الدم والزول بدخل في غيبوبة نقص السكر وده خطر حاجه عشان كده لازم نحسب كويس جرعة الانسولين

-الانسولين لازم يتحفظ في مكان بارد مثل الباجة او في حافظة مع شوية جلد او في البطيخ او في الزير

-الحقنة بتاعت الانسولين مقسمة من صغر لي مائة وحدة ما عليك الا تحسب الجرعة الداير اها من الفتيل وتتكدي انو أي واحد عنده تخين في الجلد بزاوية تسعين

-أماكن الحقن هي :-

1) البطن

b- Adrenergic symptom:-
   a- Palpitation
   b- Sweating
   c- Termer
   d- Anxiety
   e- Hunger pain

-Management :-
- If consciousness:-
   a- Given juice then meal
   b- Dissolving sugar in water then meal
- If unconsciousness:-
   a- Application of honey inside the mouth
   b- Injection of glucagon (IM)
   c- Transport to hospital

-Prevention of further attack:-
   a- Regular intake of diet
   b- Caring sugar or juice before going to school
   c- Take source of sugar always
   d- Given snake before exercise (there meal there snake)
   e- Educate the boy and other family member about hypoglycemia

-Ask her if there any question
-Thanks the mother

Focus history of neonatal jaundice

-Introduction
-Maternal disease:-
1) DM
2) Jaundice &blood group
3) Fever & rash
4) Polyhydrominas
5) Maternal drugs

-Labor condition:-
1) Term or premature
2) Mode of delivery
3) Place of delivery
4) Duration of delivery
5) Conducted by whom
6) Prolonged ROM
7) Birth asphyxia
8) Delayed cord clamping

-Neonatal condition :-
1) Cry immediately or need resuscitation
2) Start breast feeding adequately or not
3) Birth injury &birth weight
4) Blood group for neonate
5) Passage meconium

-Analysis of jaundice :-
1) Appear in first day
2) Cover whole the body
3) Color of urine &stool
4) Association:-

a. Fever
b. Convulsion
c. Vomiting
d. Refuse feeding
e. Relation of jaundice to the breast milk
f. Pallor

-F H:-

1) Sibling with jaundice
2) F H of anemia (hemolytic)
3) F H of spleenectomy
4) F H regular blood transfusion &exchange transfusion

Focus history of iron deficiency anemia

-Introduction
-Ask about adequate iron store:-

1) Maternal iron deficiency &malnutrion
2) Chronic illness
3) Tonic during pregnancy &lactation
4) Prematurity
5) Multiple pregnancy
6) Fetal blood loss from umbilical illness
7) Blood transfusion during delivery

-Inadequate intake ;-

1) Prolonged extensive breast feeding
2) Early introduction of cow milk
3) Poor intake for rich iron diet (fish, meat ,liver,egg,kidney)

-Impairment of absorption :-

A- Chronic diarrhea :
   a. Malabsorbtion
   b. Celiac disease
   c. Chronic disease

B- High phosphorous diet :-
   a. Bread
   b. Cow's milk
   c. Cereals

-Increase demand &blood loss:-

1) Bleeding (epistaxis's and other site)

2) Polyps
3) Piles
4) Passage of worm
5) Epigastric and abdominal pain

-F H & S H :-

1) Socioeconomic class
2) Income
3) School and preschool performance
4) Family size
5) Maternal age at pregnancy
6) FH of IBS polyps

Examination of pt with anemia

-Great the pt,s
-Introduce your-self
-Take permission for examination
-Explain to the informant what are you going to do
-Your position in the right side of the baby
-The baby lining flat
-Exposes the area of examination
-General look :-

1) Irritable
2) Dyspnea or not
3) Attitude
4) Dysmorphic feature

-Anthropometric measurement :-

1) Head circumference
2) Weight
3) Length

-Vital sign :-

1) Pulse (high volume pulse &tachycardia)
2) BP (low)
3) RR (increase)

-Head :-

1) Bossing (frontal &parietal)
2) Protrusion of maxilla &teeth
3) Depression of nasal bridge (indicate chronic hemolysis (medullary hemopoasis))
4) Microcephaly (fanconi anemia)

-Eyes:-

1) Pallor
2) Jaundice
3) Microophthalma(fanconi)
4) Blue sclera(IDA)

5) Retinopathy(SCA)

-Ear:-

Deafness(vitamin B_{12} deficiency)

-Mouth:-

1) Atrophic glossaitis (IDA)
2) Hyperpigmentation
3) Gingival hypertrophy(AML)

-Neck :-

- Rise JVP

-Hand:-

1) Koilonychias
2) Pallor
3) Absent thumb
4) Absent radii

-Skin:-

1) Caffe-aule,s spot
2) Hyperpigmentation
3) Vitiligo(pernicious ↓B_{12})
4) Petechial ↓purpra ↓aplastic
5) Skin ulcer (SCA)
6) Butter fly rash (SLE)

-C V S :-

1) Hemic murmur
2) Basal crepitation
3) Cardiomegaly
4) Change murmur
5) Cardiac malformation (fanconi)

-Abdomen:-

1. Tender hepatomegaly
2. Hepatosplenomegaly
    a. Mylofibrosis
    b. leukemia
    c. megaloblastic anemia
3. nephromegily :-
    a. fanconi
    b. leukemia

-C N S;-

a. Neuropathy(B_{12})
b. Ataxia(B_{12})
c. Weakness(S C A)
d. Absent knee jerk
e. +ve Babinski sign (B_{12})

-External genet ilia :-

a. To ass sexual maturity
b. Scrotal swelling (leukemia)
c. Priapism(S C A)

-Thanks the pt,s and his mother

-Cover the pt,s

-Report your finding in summary

-Give the diagnosis

focus history of generalized body swelling

introduction
Great the pt's
Take permission
Obtain pt;s name &age &residence
P/C :-
generalized body swelling duration
H/P/I:-

1. Onset
2. When it start
3. Course:-
    a. Progressive
    b. Static
    c. Regressive
4. Diurnal variation

S/R:-

1. C.P.S:-
    a. S O B &PND
    b. Cough
    c. Palpitation
    d. Epistaxis's
2. G.I.T:-
    a. Nausea
    b. Vomiting
    c. Abdominal distension
    d. Anorexia
    e. Jaundice (itching &color of the skin)
    f. Diarrhea(malnutrition)
    g. Hematemesis (portal hyper tension)
3. U.S:-
    a. Amount of urine
    b. Color of urine
    c. Frequency
    d. Pain(loin)
    e. burning micturition &suggest U. T .I)
4. C.N.S:-
    a. Headache
    b. Blurring of vision
    c. Convulsion

P M H:-

1. History of skin &urinary tract infection
2. Repeated hospitalization
3. Blood transfusion
4. Schistosomasis
5. Jaundice or contact with jaundice pt.s

Nutritional history:_
Egger to meal
Amount &frequency(to exclude malnutrition)
Vaccination history :-
Routine vaccine with BCG scar
F H :-

1. Similar condition
2. DM&HTN
3. Family size

Focus history diarrhea

Introduction:-

Analysis of diarrhea :-

1. Frequency
2. Amount
3. Content (undigested material)
4. Consistency (blood ٭mucus)
5. Charter
6. Special odor (vinger like ٭foul smelling)
7. Altered by constipation or not
8. Association :-
    a. Perianal exec oration & extremities desquamation
    b. Lesion (ulcer ٭ abscess)
    c. Mouth ulcer
9. Related to ingestion of:-
    a. Cow's milk
    b. Breast milk
    c. Bread
    d. Fruit juice
    e. Over feed
    f. Formula

Course of illness

Other GI symptom :-

1. Vomiting
2. Anorexia
3. Fever
4. Tenismus
5. Abdominal pain
6. Flatus
7. Loss of weight

Preceding infection:-

1. Ear discharge or pain
2. Chest infection

Symptoms of anemia:-

1. Pallor
2. Fatigue

P M H:-

1. Similar attack
2. Giardiasis
3. DM
4. T B
5. Thyroid
6. Liver disease
7. Connective tissue disease
8. Passage of worm
9. Food allergy
10. Lactose intolerance
11. IBS

F H:-

1. IBS
2. Cystic fibrosis

Nutritional history :-

1. Diet type
2. Over feeding
3. Hostile formula

Counseling of pt's with SCA

Introduction

Great the pt's

Introduce your-self

Explain to the pt's what are going to do

Education about the nature of the disease :-

-طفلك ده عنو انيميا منجليه وهي عبارة عن مرض وراثي الابوين حاملين للمرض وبورثو ابنائهم
-طبعا بكون في قرابة بين الابوين وبظهر في 25 % من الابناء لانو مرض متنحي و25% بكونوا سليمين اما ال50 % بكونوا حاملين للمرض فقط وبعداك لو اتزوج قريبتو ممكن يورثو ابناؤو.
- المشكله انو فى بعض الحالات الهموقلوبين بى يترسب فى كريات الدم الحمراء ودى شكل الهلال وماب يرجع يزوب تانى وبى يسبب لى نوبات.

Inquire about the PPT:

-هنالك بعض الحالات تودي الي ظهور نوبات لنيميا ذي العطش او الجفاف من الاسهال والبرد عشان كده غلبا بتظهر في الشتاء بالاضافة الي الالتهابات (الـنزلة) والصعود الي الاماكن العالية ولبس الملابس الضيقة عشان كده تنتبهو للحاجات دى.

Inquire about the crisis

-طبعا كريات الدم الحمراء بتتغير في الشكل عشان كده بتتكسر بسهولة وده بظهر في شكل شحوب مع يرقان
-كريات الدم الحمراء لما شكلها بتغير ده بخليه تتلصق مع بعضها مما يودي الي قفل الوعية الدموية ممايودي الي اللام في كل الجسم علي حسب المكان الاتقفل في الصدر البطن الاطراف المفاصل
-مرات بحصل التهاب فيروسي في نخاع العظم مما يودي الي تعطيل مصتع انتاج الخلايا الحمراء وبالتالي شحوب اللون.
-برضو في بعض الاحيان الدم بمشي الطوحال او الكبدمما يودي الي شحوب مفاجي واعراض
صدمة وعائية عشان كده انت عندك برنامج ثابت انك تتحثثي الطوحال بتاعو موجود في الجهة اليسري ل اعلي البطن

Nutritional approach

-عايزنك تركزي لينا علي البروتينات و اللحوم و البقوليات
-لازم يشرب علي طول العصائر خصوصا البرتقال
-لازم يواصل الفولك اسد عشان يساعد في زيادة تصنيع الدم
-بالنسبة للمسكنات هو يستعملها لازالة الالم كمان ميستعملها من غير ضرورة

Prophylaxis:-

لو ظهر ليو أي التهاب خاصة في الجهاز التنفسي العلوي ياهو في شكل نزلة لازم تجيبي للدكتور عشان ياخذ العلاج المناسب
لازم ياخذ كل التطعيمات الروتينية ياهو الخماسي السل شلل الاطفال الحصبة لازم ياخذ تطعيم السحائي لانو احتمال ن:شيل الطوحال سنتين لقدام
تحاولي بقدر الامكان انو تتجبني الحاحات القلناها عشان تتجنبي النوبات

Examination of pt with Down syndrome

Introduction:-
Great the pt,s
Take permission
Obtain the pt,s name age
General look :-

1. Dysmorphic change
2. Pallor (autoimmune)
3. Tachypnea
4. Sign of respiratory distress

Vital signs:-

1. Pulse increase(anemia infection)
2. BP
3. RR
4. Temp

Anthropometric measurement:-

1. HT
2. WT FTT

Head :-

1. Microcephaly
2. Brachycephalic(flat occiput)
3. White anterior fontanel
4. Sickly hair

Eyes :-

1. Hypertolrisim
2. Oblique balberal fissure(staining eye)
3. Epicanthi fold
4. Few lashes
5. Apparent sequent

ear :-

1. Small
2. Over folded helix
3. Low sit year
4. Ear discharge

Nose:-

1. Short
2. Depress nasal bridge
3. Nasal discharge

Mouth :-

1. Protruded tongue
2. Fissured tongue
3. Delayed teeth eruption

Neck:-

1. Short
2. Broad
3. Excess skin posteriorly

Hand :-

1. Short
2. Broad
3. Semian cris
4. Little finger :-
    a. Curved
    b. Radiomentery 2nd
    c. Single pharyngeal crisis

Feet:-

1. Large space between finger
2. 2nd toe longitudinal cress

Chest :-
infection
C.V.S :-

- ✓ Congenital heart disease(AVSD)

Abdomen:-

1. Malformation
2. Distended abdomen
3. Umbilical hernia
4. Imperforated anus
5. Hepatomegaly

Genetilia :

1. Poorly develop
2. Delayed sexual dysfunction
3. Hypo or epispedia

Thanks the informant
Cover the pt,s
Giving your difindin

Neonatal resuscitation

Step zero:

Birth >>> APGAR score

Assess the following :-

1. GA
2. Breathing and crying
3. Good muscle tone
4. Pink color
5. Clear of meconium

If the above is yes >>>Routine care :-

1. Provide warmth
2. Clear the air way
3. Drying.

If the above is no>>>start the resuscitation :-

Step one:-

1. Provide warmth
2. Clear the air way
3. Dry stimulate respiration
4. Give oxygen

Then evaluate the following:-

a. Respiration
b. HR
c. Color

If there:-

a. Breathing adequately
b. HR more than 100
c. Pink color

Do supportive care:-

a. Radiant warmer
b. Frequent evaluation

If there:-

a. Apnea
b. HR less than 100
c. Color not became pink

>>>>Second step:-

Step two:- {Positive pressure ventilation }

Criteria of bag used of ventilation :-

a. Size no large than 750 ml
b. Ability to deliver 90-100% oxygen
c. Ability to avoid excessive pressure
d. Approximate size of mask
e. Size of breath (5-8ml/kg)
f. Breathing size tidal volume 1/3 of amount of gas in the bag

Procedure:-

a. Position (your-self)at the bay head and bed size
b. Rate 40-60 breath/minute
c. Breath 2or3

If the:-

a. Adequate ventilation
b. HR 100
c. Pink color

Ongoing care:- {At NICC}

a. Temperature control
b. Monitoring:-
    a. BP
    b. O2 saturation
    c. HR
    d. Lab monitoring
    e. RBG
    f. ABG
    g. Anticipate the complications

If HR less than 60 transfer to next step

Step three:- {Positive ventilation pressure and chest compression}

How can do it :-

1) Need two persons

2) Compress the heart against spine at the level of xiphoid between the two nipple by thumb or tow finger
3) Flexion at the first joint and press vertically
4) Depth 1/3 of anterio-posterior diameter
5) Don't lift your hand
6) Rate (1,2,3)breath in 2 second or 90:30/minute

If there is a problem in ventilation consider endotracheal tube

Indication of endotracheal tube:-

1) If PPV not result in a good chest rise
2) To facilitate coordinated chest compression
3) To give :-
    a. Epinephrine
    b. Surfactant
4) Suction of meconium
5) Extreme pre mature
6) Suspected diaphragmatic hernia

Endotracheal tube:-

1) Length =weight by 6
2) Size :-
    a. 2.5 wt <1 kg or GA <28 week
    b. 3 wt 1-2kg or GA 28-34 week
    c. 3.5 wt 2-3 kg or 34-38 week
    d. 3.5-4 wt >3kgor GA >38week

How to introduce the ETT :-

1) Position in sniffing
2) Hold laryngoscope by your left hand
3) Visualization :-
    a. Stabilize the head by right hand
    b. Slide the laryngoscope over the right tongue
    c. Lift the blade of laryngoscope
    d. Apply the pressure to cricoid cartilage and suction
    e. Insert the tube and stabilize it
4) Check the tube insertion :-
    a. Rise the chest
    b. No gastric distention
    c. Breathing on both lung field

If suspected not in the trachea:-

1) Remove it
2) Bag and mask
3) Repted the intubation

How to minimize the hypoxia during intubation :-

1) Pre-oxygenation
2) Deliver free flow oxygen during intubation
3) Limit the attempt to 20 second

If the HR >60 retain to next step if the HR <60 go to next step

Fourth step

Administer epinephrine

Rout :-

Can be given through :-

1) ETT
2) Umbilical vein
3) Dose 0.1-0.3ml/kg

Can be repeated every 3-5 minute

Percussions of use :-

1) There should be good chest movement

2) Chest compression are given
3) ETT should be in place
4) Drug reach the baby heart

Other indication :-

1) $NAHCO_3$
   a. Given through umbilical vein ue
   b. 2mg/kg rate 1mg/kg/mint
2) Volume expander:-
   a. With normal saline or ringer lactate or o-ve blood
   b. 10 ml/kg through umbilical vein of rate 5-10ml/minute

Then stop resuscitation after 15 minute

At which time we doesn't initiate neonatal resuscitation?

1) GA <23 weeks
2) Weight <400 grams
3) Ancephlay
4) Trisomy 13 (pat au) trisomy 18 Edward

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